جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : اميره عبد المنعم امين عبد المنعم
المحافظة : الجيزه **الحي / المدينة :** العجوزه **العنوان :** ----
رقم البطاقة : 28207240104741 **رقم التليفون :** 01003106682 **تاريخ تسجيل الإستمارة :** 10-10-2026
العمر(تاريخ الميلاد) : 24-07-1982 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: اميره عبد المنعم امين عبد المنعم
National ID: 28207240104741

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

١٢٢٠

Name : Amira abd el moneem ameen
Branch : Portable
Ref.Physician Prof : ct cap+c
Date : 10-02-2026 01:18
Medical No. : GRI-54825
Ins.Company :

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Known case of breast CA. no available previous films for comparison.

I- CHEST:

- Small right middle lobe pulmonary nodule measuring 4 mm.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- Few subcentimetric paracardiac lymph nodes.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- Few subcentimetric para-aortic and bilateral external iliac lymph nodes.
- No ascites.

Opinion:

❖ **Small right middle lobe pulmonary nodule.**

Dr. Sherif Wagdy

Dr. Sherif Wagdy

CT Scan
[DOCTYPE:CT_SCAN]

Name:	**اميرة عبد المنعم امين**
MR. :	**1220**

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

The patient was initially diagnosed with de novo metastatic breast cancer to the lung. She underwent upfront surgery prior to the discovery of pulmonary metastases and subsequently received 11 weeks of taxane (taxol) plus intravenous trastuzumab (herceptin), which was discontinued because of severe neuropathy. She is now on endocrine therapy (ET) combined with intravenous trastuzumab; the current trastuzumab cycle is number 21, and a dose of zoladex was due on 3 March 2025.

The patient has a history of left-sided breast cancer treated with breast-conserving surgery (BCS) in April 2024. Post-operative imaging revealed pulmonary nodules. She received adjuvant taxol and trastuzumab, followed by hormonal therapy with letrozole (Femara) and a gonadotropin-releasing hormone agonist (Zoladex) initiated in October 2024 for trastuzumab 440 mg and Femara.

Zoladex was scheduled for administration on 3 March 2025 in conjunction with trastuzumab intravenous therapy pending new echocardiogram and computed tomography (CT) assessments.

The patient underwent BCS in April 2024 before the detection of lung metastasis and was subsequently placed on letrozole (Femara) and intravenous trastuzumab. Small subcentimetric pulmonary nodules were noted. Zoladex was due on 3 March 2025 while awaiting approval; an echocardiogram performed on 20 January showed a left ventricular ejection fraction of 60 %. The patient was on trastuzumab cycle 24 with letrozole and Zoladex, and on the day of the note she was receiving trastuzumab cycle 25 and preparing for cycles 26, with pharmacy providing trastuzumab, letrozole, supportive therapy, and Zoladex. A CT on 10 February 2026 demonstrated a small right middle-lobe pulmonary nodule measuring 4 mm.

Pharmacy records on 26 March 2026 indicated the patient's weight at 80 kg. An echocardiogram on 20 January showed an ejection fraction of 60 %. The patient received trastuzumab 440 mg (cycle 25), letrozole, and Zoladex 10.8 mg, along with supportive treatment.

The patient's premenopausal left-breast cancer was initially staged as T2 N1 (11/11) with estrogen-receptor (ER) 8/8, progesterone-receptor (PR) 8/8, and HER2-positive disease. After BCS in April 2024, she received adjuvant taxol and trastuzumab, followed by Zoladex, letrozole, and continued trastuzumab. A CT in June 2022 showed bilateral pulmonary nodules, the largest measuring 9 mm; a subsequent CT in February 2026 demonstrated a 5 mm nodule. A repeat echocardiogram in October 2025 was pending for planning trastuzumab cycle 26 (440 mg) with letrozole and Zoladex, with the next Zoladex dose due on 18 June.

On 3 June 2026 the patient presented for trastuzumab cycle 27. She was in good general condition and continued supportive therapy.

On 5 July 2026 the patient received trastuzumab cycle 28 (440 mg) together with letrozole and Zoladex, with the next Zoladex dose scheduled for 22

تحريرا في ,

التاريخ :	**18-09-2026**	**الوقت :**	

الطبيب المعالج : **dina makram**

يعتمد ,

June; the echocardiogram showed an ejection fraction of 65 %.

On 26 July 2026 the patient has metastatic breast cancer with subcentimetric pulmonary nodules. A CT chest/abdomen/pelvis performed in July 2026 showed complete resolution of the previously noted small right middle-lobe pulmonary nodule, indicating no radiographic evidence of metastasis. She received intravenous trastuzumab in the day-care pharmacy on the day of the visit.

atient Name: Amira Abd Al Moneim Ameen bd Al Moneim	Date : 29/12/2024
kam : C.T CHEST	Age : 42 Year(s)
edical ID: 1488/2024	Pat. ID: 54825

- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

III-The scanned skeleton:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

PINION:

Mild regression of the left breast lesion and associated derlying muscle thickening.

Otherwise; Stationary course of the rest of the study.

Dr. Shimaa Hussien Ali

Dr. Walid Mohamed, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name	: الأستاذه / اميره عبدالمنعم امين عبدالمنعم	Request No.	: 13009467
Patient No.	: 28207240104741	Request Date	: 19/05/2024
Path No.	: SR-637/24	Verification Date	: 12/06/2024
Sex	: Female	Financial Category	: دار السلام - نفقة الدولة
Age	: 41 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hega

Pathology Report

Clinical Data: Left breast mass (WLE & Axilary clerance).
Nature of Specimen: Slide Revision

Gross Examination:
- 10 paraffin blocks labeled by (2409635) and 13 slides were received.

Microscopic Examination:
- Histological type: Invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
- DCIS: No.
- Angioinvasion: No.
- Perineural invasion: No.
- Lymph node involvement: Positive one metastatic lymph node (1/11).
- IPT:
- ER: Positive nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Positive nuclear reaction in 80% of tumor cells (score 8/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 40% nuclear reaction of tumor cells.

Diagnosis:
Left breast mass, Slide revision:
Invasive duct carcinoma, Grade II.
Free recieved margins.
Positive one metastatic lymph node (1/11).
IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Positive (score 3+).

Verified by :
Dr: Sagda El-Kady

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الاستاذه / اميره عبدالمنعم امين عبدالمنعم	Request No.	: 13009467
Patient No.	: 28207240104741	Request Date	: 19/05/2024
Path No.	: SR-637/24	Verification Date	: 12/06/2024
Sex	: Female	Financial Category	: م - نعمة الدولة
Age	: 41 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada

Pathology Report

- Ki67: Positive in 40% of tumor cells

Signature
Dr. Marwa Mahmoud.
Dr. Sagda Elkady.

Verified by :
Dr. Sagda El-Kady

غرفة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. 14) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, I

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name: Amira Abd Al Moneim Ameen Abd Al Moneim	Date : 29/12/2024
Exam : C.T CHEST	Age : 42 Year(s)
Medical ID: 1488/2024	Pat. ID: 54825

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

- **Reported case of left breast cancer with CBS.**
- **Compared to last study dated 21-9-2024, the current study revealed:**

I -Chest:

- **Regressive course regarding the:**
  - Left breast heterogeneous iso-intermediate parenchymal density and the thickening of the overlying skin.
  - Previously seen irregular ill-defined heterogeneous density seen at the lateral aspect of left pectoral muscles showing heterogeneous soft tissue density with surrounding fibrotic bands.
- ***Stationary course*** The previously noted few bilateral mostly subpleural small and tiny pulmonary nodules with the largest seen at lateral segment of the middle lung lobe still measuring about 0.6. No newly developed nodules.
- ***Stationary course*** regarding the previously noted prominent subcentimeteric axillary and supraclavicular lymph nodes.
- No newly developed lesions.
- No pleural or pericardial effusion.
- Patent trachea and major bronchi.
- Normal non-contrast CT appearance of the heart and major thoracic aorta.

II-Abdomen and pelvis:

- The liver is average in size with homogenous low CT parenchymal attenuation and regular outline. No definite hepatic focal lesions,
- No intrahepatic or extrahepatic biliary dilatation.
- Normal size & parenchymal texture of the spleen with nc focal lesions seen.
- No pathological enlarged abdominal or pelvic lymph nodes.

CT Scan
[[DOCTYPE:CT_SCAN]]

[[DOCTYPE:MDT_FORM]]

MDT form

A-Patient ID:

1- Name : امیره عبدالمنعم امین عبدالمنعم Sex: Female

3- Medical number: 83206398

Old Med : 1488-2024

2-ID number:

Patient Age : 42 Y

Family history		Phone	01003106682	BMI	REASON OF M
marital status		NRS Score		GROUP	Date of MDT
Co morbidity		Menopausal status		Initial STAGE	

Clinical data

Breast Imaging :

Mets Workup :

GX	ER	PR	Her 2 Neu	SISH
	8/8	8/8	3	

T	N	M
		1 lung

Sonomammography

PATIENT HISTORY

pt underwent lt. BCS (4-2024) before discovery of lung metastasis
denovo metastatic breast cancer to lung (CT chest: 12-6-2024)
pt on femara+herceptin

MDT OUTCOME & PLAN

Pt finished 11 weeks of taxol + 3 cycles Herceptin However developed PN GR III.
CTs: Regression
to continue on Herceptin and ET (AI+ zoladex)

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	